

Product Requirements Document

Clinic Appointment & Intake System

1. Product Summary

The Clinic Appointment & Intake System enables patients to schedule appointments, complete pre-visit intake forms, receive appointment reminders, and allow clinic staff to review appointment and intake information before the visit.

The system is intended for outpatient clinics that want to reduce phone-based scheduling, decrease front-desk paperwork, improve visit readiness, and reduce manual data entry into their clinical systems.

2. Product Goals

The product should:

1. Allow patients to request, schedule, reschedule, and cancel appointments.
2. Allow patients to complete required intake forms before their visit.
3. Reduce manual administrative work for clinic staff.
4. Improve clinic visibility into upcoming appointments and intake completion status.
5. Send timely appointment confirmations and reminders.
6. Support integration with the clinic's electronic health record system.
7. Maintain appropriate privacy, auditability, and reliability for patient-related workflows.

3. Non-Goals

The product will not:

1. Replace the clinic's electronic health record system.
2. Provide clinical diagnosis, triage, or treatment recommendations.
3. Manage provider credentialing, payroll, or clinical staffing.
4. Process insurance claims or patient billing in the initial release.
5. Store a complete longitudinal medical record.
6. Replace clinical documentation performed by providers during or after the visit.

4. Target Users

User	Description
Patient	Schedules or requests appointments, completes intake forms, receives reminders, and updates basic visit

User	Description
Clinic Staff	Manages appointment requests, reviews intake completion, updates appointment status, and assists patients.
Provider	Reviews relevant intake information before the patient visit.
Clinic Administrator	Configures appointment types, intake requirements, reminder rules, and staff access.

5. External Systems

External System	Description
Electronic Health Record System	Receives appointment and intake data and may provide patient, provider, or appointment reference information.
Notification Provider	Sends email and SMS confirmations, reminders, and status notifications.
Insurance Eligibility Service	Optional service used to check basic insurance eligibility before a visit.
Identity Provider	Optional system used to authenticate staff and provider users.

6. Primary User Problems

Patients often need to call during office hours to schedule, reschedule, or cancel appointments. Intake forms are frequently completed at the clinic shortly before the visit, causing delays and additional administrative effort.

Clinic staff often need to manually enter appointment and intake information into existing clinical systems. Providers may not have enough time to review patient-provided information before the visit. Reminder workflows are often inconsistent, leading to missed appointments and operational inefficiency.

7. Product Capabilities

7.1 Appointment Scheduling

Patients should be able to:

1. Search for available appointment slots.
2. Filter by appointment type, location, provider, or date range where supported.
3. Request or directly book an appointment depending on clinic policy.
4. Receive confirmation after the appointment is requested or booked.

5. Reschedule or cancel an appointment according to clinic rules.
6. View appointment instructions, location, date, and time.

Clinic staff should be able to:

1. View upcoming appointments.
2. Review appointment requests.
3. Confirm, reject, reschedule, or cancel appointments.
4. Override scheduling restrictions when authorized.
5. Add internal notes that are not visible to patients.

7.2 Digital Intake Forms

Patients should be able to:

1. Complete required intake forms before the appointment.
2. Save partial progress and return later.
3. Submit demographic, contact, medical history, medication, allergy, consent, and visit-reason information.
4. Upload supporting documents where enabled.
5. Receive confirmation after successful submission.

Clinic staff and providers should be able to:

1. View intake completion status.
2. Review submitted intake information.
3. Identify missing required information.
4. Mark intake information as reviewed.
5. Request corrections or additional information from the patient.

7.3 Appointment Confirmations and Reminders

The system should:

1. Send appointment confirmations.
2. Send appointment reminders based on clinic-configured timing.
3. Support email reminders in the initial release.
4. Support SMS reminders where enabled.
5. Include date, time, location, and preparation instructions.
6. Avoid exposing sensitive clinical information in reminder messages.
7. Track delivery status where supported by the notification provider.

7.4 Electronic Health Record Synchronization

The system should:

1. Send confirmed appointment information to the clinic's electronic health record system.
2. Send submitted intake information to the electronic health record system where configured.
3. Store synchronization status.
4. Retry failed synchronization attempts when appropriate.
5. Show staff whether appointment or intake synchronization succeeded, failed, or is pending.
6. Preserve enough detail for staff to troubleshoot synchronization issues.

The electronic health record system remains the source of truth for the complete clinical record.

7.5 Insurance Eligibility Check

Where enabled, the system should:

1. Collect required insurance information during intake.
2. Submit eligibility requests to an external eligibility service.
3. Display eligibility status to authorized clinic staff.
4. Record eligibility check date, status, and result summary.
5. Allow the clinic to configure whether eligibility failure blocks scheduling, flags the appointment, or requires staff review.

8. Representative User Journeys

8.1 Patient Schedules an Appointment

1. Patient opens the scheduling experience.
2. Patient selects appointment type and preferred date range.
3. System displays available appointment options.
4. Patient selects an appointment slot.
5. Patient enters required identity and contact information.
6. System creates an appointment request or confirmed appointment.
7. Patient receives confirmation.
8. Clinic staff can view the appointment.
9. Appointment information is synchronized with the clinic's electronic health record system.

8.2 Patient Completes Intake

1. Patient receives an intake link after appointment confirmation.
2. Patient verifies identity or signs in.
3. Patient completes required intake sections.
4. System validates required fields.
5. Patient submits the intake package.

6. System stores the submitted intake information.
7. Clinic staff and providers can review the intake before the visit.
8. Intake information is synchronized with the electronic health record system where configured.

8.3 Staff Reviews Upcoming Appointments

1. Clinic staff opens the appointment dashboard.
2. Staff filters appointments by date, provider, or status.
3. Staff reviews intake completion status.
4. Staff identifies appointments with missing intake information.
5. Staff sends a reminder or contacts the patient.
6. Staff updates appointment status where needed.

8.4 Appointment Reminder Is Sent

1. System identifies an upcoming appointment requiring a reminder.
2. System checks patient communication preferences.
3. System prepares reminder content.
4. System sends the reminder through the notification provider.
5. System records reminder delivery status where available.
6. Staff can view reminder history if troubleshooting is needed.

9. Functional Requirements

ID	Requirement
FR-001	The system shall allow patients to view available appointment slots.
FR-002	The system shall allow patients to request or book appointments.
FR-003	The system shall allow patients to reschedule or cancel appointments according to clinic rules.
FR-004	The system shall allow staff to review and manage appointment requests.
FR-005	The system shall support configurable appointment types.
FR-006	The system shall support digital intake form completion.
FR-007	The system shall allow patients to save incomplete intake forms and return later.
FR-008	The system shall validate required intake fields before submission.
FR-009	The system shall store submitted intake

ID	Requirement
	forms.
FR-010	The system shall allow staff and providers to review submitted intake information.
FR-011	The system shall send appointment confirmations.
FR-012	The system shall send appointment reminders based on configurable rules.
FR-013	The system shall integrate with an external notification provider.
FR-014	The system shall synchronize confirmed appointment data with the electronic health record system.
FR-015	The system shall synchronize submitted intake data with the electronic health record system where configured.
FR-016	The system shall record synchronization status and failure details.
FR-017	The system shall support retry handling for failed external integrations.
FR-018	The system shall support optional insurance eligibility checks.
FR-019	The system shall maintain an audit trail for appointment, intake, reminder, and synchronization events.
FR-020	The system shall enforce role-based access for staff, providers, and administrators.

10. Non-Functional Requirements

Category	Requirement
Privacy	The system shall protect patient information using appropriate access control, encryption, and least-privilege authorization.
Security	Staff, provider, and administrator access shall require authenticated sessions and role-based authorization.
Auditability	The system shall record key actions involving appointment changes, intake submissions, staff review, reminders, and

Category	Requirement
	external synchronization.
Availability	Patient scheduling and intake should be available outside clinic business hours.
Reliability	Failed external integrations shall be visible, recoverable where appropriate, and not silently discarded.
Usability	Patient-facing workflows should be usable on desktop and mobile browsers.
Accessibility	Patient-facing screens should support accessible navigation and form completion.
Performance	Appointment search, form save, and form submission should respond quickly under normal clinic load.
Configurability	Clinics should be able to configure appointment types, reminder timing, intake form requirements, and cancellation rules.
Data Retention	Appointment, intake, and audit data shall follow clinic-defined retention policies.

11. Data Requirements

The system should manage the following data:

1. Patient identity and contact details.
2. Appointment records.
3. Provider and clinic location references.
4. Appointment type configuration.
5. Intake form definitions.
6. Intake form drafts.
7. Intake form submissions.
8. Insurance eligibility request and result summaries.
9. Reminder and notification status.
10. Electronic health record synchronization status.
11. Staff notes.
12. Audit events.
13. User roles and permissions.

The system should avoid storing unnecessary clinical data beyond what is required for scheduling, intake, review, and synchronization.

12. Reporting and Operational Visibility

Clinic staff and administrators should be able to view:

1. Upcoming appointments.
2. Appointment status by date, provider, or location.
3. Intake completion status.
4. Appointments missing required intake information.
5. Reminder delivery history.
6. Electronic health record synchronization status.
7. Failed integration attempts requiring attention.
8. Basic operational metrics such as appointment volume, intake completion rate, and cancellation rate.

13. Success Metrics

Metric	Target Direction
Appointment requests completed without phone call	Increase
Intake forms completed before visit	Increase
Front-desk intake time	Decrease
Appointment no-show rate	Decrease
Manual re-entry of appointment and intake data	Decrease
Failed synchronization events unresolved after one business day	Decrease
Patient scheduling completion rate	Increase
Staff time spent tracking incomplete intake forms	Decrease

14. Risks and Open Questions

Area	Question or Risk
EHR integration	Which electronic health record systems must be supported first?
Scheduling authority	Is the system allowed to directly book appointments, or only submit appointment requests?
Patient identity	How should patients verify identity before viewing or submitting appointment-related information?
Data ownership	Which data is authoritative in this system

Area	Question or Risk
	versus the electronic health record system?
Notification consent	How will communication preferences and SMS consent be captured?
Intake customization	How much intake form customization is required by clinic, specialty, provider, or appointment type?
Eligibility handling	Should failed or unavailable insurance eligibility block scheduling or only flag staff review?
Downtime behavior	What should staff and patients see when external systems are unavailable?
Retention	How long should intake drafts, submitted forms, reminders, and audit records be retained?

15. Release Scope

MVP

The MVP should include:

1. Patient appointment search and request or booking.
2. Staff appointment review and status management.
3. Basic intake form completion and submission.
4. Email appointment confirmations.
5. Email appointment reminders.
6. Electronic health record appointment synchronization.
7. Staff visibility into synchronization status.
8. Role-based access for staff and providers.
9. Audit logging for major appointment and intake actions.

Post-MVP

Future releases may include:

1. SMS reminders.
2. Insurance eligibility checks.
3. Advanced intake form builder.
4. Provider-specific intake templates.
5. Patient document upload.
6. Patient self-service profile updates.
7. Waitlist management.
8. Bidirectional electronic health record updates.

9. Multi-location scheduling optimization.
10. Analytics dashboard for no-shows and intake completion.
11. External identity provider integration.
12. Advanced retry and reconciliation tooling.